

# Necrotizing Fasciitis/Myonecrosis

Necrotizing fasciitis is an aggressive soft-tissue infection that requires rapid diagnosis, early surgical intervention, frequent operative debridements, and soft-tissue reconstruction. It is rare but has a very aggressive course (see Chap. 179).

Myonecrosis and rhabdomyolysis have been described after prolonged surgery. Initially, the disorder presents with an area of buttock skin breakdown, diagnosed as a simple decubitus ulcer, but it then develops into extensive myonecrosis leading to renal failure that necessitates dialysis.

## Other Complications

Among elderly bedridden patients who have surgery for pressure ulcers, the mortality rate is high (68 percent in one series). Other postsurgical complications include hematoma, seroma, wound dehiscence, abscess formation, sepsis, and death. Death has been reported to occur during acute hospitalization in 67 percent of patients who develop a pressure ulcer compared with 15 percent of at-risk patients without pressure ulcers. The risk does not appear to correlate with the severity of the pressure ulcer.

Other less commonly reported complications of pressure ulcers include amyloidosis, endocarditis, meningitis, septic arthritis, myiasis, pseudoanecrosis formation, and gangrene.

FIGURE 99-6 Algorithm for the approach to a patient with pressure ulcer.

TABLE 99-2  
Components of Three Risk Assessment Scales for Pressure Ulcers

| RISK FACTOR | NORTON | WATERLOW | BRADEN |
|-------------|--------|----------|--------|
| Mobility    | Yes    | Yes      | Yes    |

| RISK FACTOR                      | NORTON | WATERLOW | BRADEN |
|----------------------------------|--------|----------|--------|
| Activity level                   | Yes    | No       | Yes    |
| Nutritional status               | No     | Yes      | Yes    |
| Mental status                    | Yes    | No       | Yes    |
| Incontinence/moisture conditions | Yes    | Yes      | Yes    |
| General physical condition       | Yes    | Yes      | No     |
| Skin appearance                  | No     | Yes      | No     |
| Medication                       | No     | Yes      | No     |
| Friction and shear               | No     | No       | Yes    |
| Weight                           | No     | Yes      | No     |
| Age                              | No     | Yes      | No     |
| Specific predisposing diseases   | No     | Yes      | No     |
| Prolonged pressure               | No     | Yes      | No     |

# PROGNOSIS

The prognosis of pressure ulcers depends on the cause. In paraplegic patients, 50 percent to 70 percent of pressure ulcers heal with non-surgical intervention. However, in the long-term care setting in which patients may have multiple medical problems, the prognosis is worse, and complications such as sepsis, osteomyelitis, amyloidosis, anemia, and malignancy can develop.

The age-adjusted mortality rate for pressure ulcers has been calculated as 3.79 deaths per 100,000 population. The majority of pressure ulcer-associated